



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ولاء محمد عبدالمعطي زهران
المحافظة : المنوفية الحي / المدينة : أشمون
رقم البطاقة : 27907230106425 رقم التليفون : 01273135042
العمر (تاريخ الميلاد) : 23-07-1979 المهنة : ربه منزل
العنوان : محلة سبك
تاريخ تسجيل الإستمارة : 20-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج كيماوي تاكسان (دوسيتاكل)
قسم العلاج :
الأورام
[14152]
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: ولاء محمد عبدالمعطي زهران
National ID: 27907230106425



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Docetaxel chemotherapy

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقق الشخصية

ولاء
محمد عبد المعطي زهران
محلة سيك
مركز اشمون - المنوفية

١٩٧٩/٠٧/٢٣ ٢٧٩٠٧٢٣٠١٠٦٤٢٥
IX3227568

Dms
زليودا

٢٠٢١/٠٩ ٢٧٩٠٧٢٣٠١٠٦٤٢٥
حاصلة على دبلوم تجارة -
أنثى مسلمة
خالد احمد محمد ابو علي
البطاقة سارية حتى ٢٠٢٨/٠٩/٢٠ ١٢

متزوجة



Age: 46 Years (s), 9 Months (s)

Patient Name: WALAA MOHAMMED
ABDLMOEETY ZAHRAH

Sex: Female

Procedure: Mri Brain Routine

Report Date: 26/04/2026 06:34:44

Modality Name: MR

MRI OF THE BRAIN

MRI TECHNIQUE:

- Sagittal: T1WI 6 mm sections (Plate 1).
- Axial: T2 & FLAIR WI 5 mm sections (Plates 2 & 3).
- Coronal: T1WI images 5 mm sections (Plate 4).
- Post Gd- DTPA sagittal, axial & coronal T1 WI (Plates 5, 6 & 7).

MRI FINDINGS:

- One left frontoparietal and two right temporoparietal intraaxial small ill-defined heterogeneously enhancing likely metastatic SOLs measuring 2.2x2.1x1.6 , 1.6x1.1 and 0.9x0.9 cm respectively seen isointense to the cerebral cortex in T1, T2 and FLAIR with no evidence of corresponding diffusion restriction
- The lesions are seen surrounded by grade II vasogenic edema which shows relative diffusion restriction.
- The lesions and the edema on either side are seen creating mass effect in the form of effacement of the surrounding sulci however additional cerebral edematous changes and mild compression of the ipsilateral lateral ventricle are seen on the left side only.
- Right frontoparietal small ill-defined foci of high T2/FLAIR signals involving the deep white matter
- Normal size and parenchymal signal intensity pattern of the cerebellum, and brain stem.
- Normal size, shape and position of the ventricular system. No signs of hydrocephalus or atrophy
- No shift of the midline structures.
- No extra-axial collections.

OPINION:

- Bilateral few intraaxial brain metastatic SOLs
- Chronic small vessel disease.

Dr/ Shereen Reda Tayel(MD)



Patient ID: 0000_6*161	Patient Name: WALAA MOHAMMED ABDLMOEETY ZAHRAH
Age: 46 Years (s), 9 Months (s)	Procedure: Mri Brain Routine
Sex: Female	Modality Name: MR
Report Date: 26/04/2026 06:34:44	

MRI OF THE BRAIN**MRI TECHNIQUE:**

- Sagittal: T1WI 6 mm sections (Plate 1).
- Axial: T2 & FLAIR WI 5 mm sections (Plates 2 & 3).
- Coronal: T1WI images 5 mm sections (Plate 4).
- Post Gd- DTPA sagittal, axial & coronal T1 WI (Plates 5, 6 & 7).

MRI FINDINGS:

- One left frontoparietal and two right temporoparietal intraaxial small ill-defined heterogeneously enhancing likely metastatic SOLs measuring 2.2x2.1x1.6 , 1.6x1.1 and 0.9x0.9 cm respectively seen isointense to the cerebral cortex in T1, T2 and FLAIR with no evidence of corresponding diffusion restriction
- The lesions are seen surrounded by grade II vasogenic edema which shows relative diffusion restriction.
- The lesions and the edema on either side are seen creating mass effect in the form of effacement of the surrounding sulci however additional cerebral edematous changes and mild compression of the ipsilateral lateral ventricle are seen on the left side only.
- Right frontoparietal small ill-defined foci of high T2/FLAIR signals involving the deep white matter
- Normal size and parenchymal signal intensity pattern of the cerebellum, and brain stem.
- Normal size, shape and position of the ventricular system. No signs of hydrocephalus or atrophy
- No shift of the midline structures.
- No extra-axial collections.

OPINION:

- Bilateral few intraaxial brain metastatic SOLs
- Chronic small vessel disease.

Dr/ Shereen Reda Tayel(MD)

Patient Name : **الانسانى، زهره**
Patient No. : **127907230106426**
ID No. : **14604/24**
Sex : **Female**
Age : **46 Y**
Referring Doc :

Request No. : **13009906**
Request Date : **25/11/2024**
Verification Date : **11/12/2024**
Financial Category : **المرضى - برنامج دار السلام**
Department : **Out Patient**
Registered by : **Mrs. Mayada Haggazy**

Pathology Report

Clinical Data: Right breast cancer, post NACT.

Nature Specimen: Right modified radical mastectomy specimen.

Gross Examination:

- * Procedure: Right MRM measured 24x21x8 cm with covering edematous skin oedema measured 21x15 cm.
- * Tumor extent: A greyish white rubbery to firm tumor bed site.
- * Size: 12x11x3 cm.
- * Skin involvement: Nipple & areola were unremarkable grossly.
- * Distance from margins: 1 cm away from deep margin and 1.5 cm away from skin.
- * Lymph nodes number and size: Attached axilla measured 6x6x2 cm, containing 8 LNs up to 2.2x1 cm.

Designation:

- 1-12: The tumor bed site.
- 13: The deep margin.
- 14: The nipple-areola complex.
- 15-16: The axillary lymph nodes.

Grossing by: Dr. Yasmin Elssam.

Microscopic Examination:

- * Histological type: Residual invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Maximal tumor diameter 1x1 cm
- * Actual tumor cellularity: 3%, tumor necrosis index: 97%
- * Ductal carcinoma in situ: Extensive DCIS of high grade (90%).
- * Skin involvement: Free nipple and areola skin.

Verified by :

Dr. Yoka Mohamed

Printing Date : 11/12/2024 16:39:35

Page 1 of 1

Printing By : amrwa mohamed yassir



Room No. (1) - First Floor - Building No. 987, Nile Corniche - Mar El Qadima - Cairo, Egypt

Request No. : 13009905
Request Date : 25/11/2024
Verification Date : 11/12/2024
Financial Category : Out Patient
Department : Mrs. Mayada Hegazy
Registered by :
Referring Doc :
No. : 27907230103425
No. : 4604/24
Sex : Female
Age : 45 Y

Request No. : 13009905
Request Date : 25/11/2024
Verification Date : 11/12/2024
Financial Category : Out Patient
Department : Mrs. Mayada Hegazy
Registered by :
Referring Doc :
No. : 27907230103425
No. : 4604/24
Sex : Female
Age : 45 Y

Pathology Report

- Lymph node involvement: Involved with mets in 1 out of 8 lymph nodes.
- Resection Margins: Free deep resection margin.
- Histologic response to therapy: Partial pathological response.
- Microcalcifications: Positive.

Diagnosis:

- Right breast cancer post NAQT, MRM specimen:
- Residual invasive duct carcinoma, Grade II, with extensive DCIS of high grade (90%).
- Maximal tumor diameter: 1 cm
- Actual tumor cellularity: 3%, tumor necrosis index: 97%.
- Free deep resection margin.
- Free nipple and areola skin.
- Lymph nodes are involved with mets in 1 out of 8 lymph nodes.
- Partial pathological response to therapy.
- Stage: ypT1, ypN1.
- RCB: 1.977, RCB class II.

Signature
Dr. Rana Fathy.
Dr. Toka Medhat.

Verified by :
Dr. Toka Medhat

Issuing Date: 11/12/2024 10:39:38

Page 1 of 1

Printing by : mayya mohamed youss

Room No. (4) - First Floor - Building No. 987, Nile Corniche - Al-Haram - Cairo, Egypt



MDT FORM

25/11/2025 G

Name	ولاء محمد عبد الله	MRN	1720/000
Age	45y	ID	09000000000000000000
Menopausal status	prey 0	Phone	01273135042
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDCII			R	ps:1	L
ER:	PR:	HER 2:	KI67:			
-u	-u	3+				
Sonomammography:						
Mets Workup:				premy 0 pr Br = CTy b.N. Mo will rest Br TNM Rec MNT TCHP pr MNT 27/11/2024:- 4pT0 4pN1 1/2 Rec 124e Kadye Est 4/11/2024		
Recommendations:				Relly heath (By you) MRI Brain: di 2/11/2024 Bilal 50L incerebemi CT: free RT middle lobe pul.		
MDT OUTCOME & PLAN				① for SRS tobi (GK.) ② Tykabo / Xeloda.		

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
محمد عبد الله					

جمهورية مصر العربية
بطاقة تحقيق الشخصية

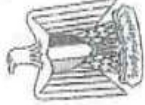


ولاء
محمد عبد المعطي زهران
محلة سبك
مركز أشمون - المنوفية

١٩٧٩/٠٧/٢٣

٢٧٩٠٧٢٣٠١٠٦٤٢٥

IX3227568



٢٠٢١/٠٦ ٢٧٩٠٧٢٣٠١٠٦٤٢٥
حاصلة على نيلوم تجارة

متزوجة
أنثى
مسلمة
خالد احمد محمد ابو عمر
البطاقة سارية حتى ٢٠٢٨/٠٦/٢٠

١٢





Patient Name: WALAA MOHAMMED ABDLMJOEETY ZAHRAH	
Age: 46 Year(s), 9 Month(s)	Procedure: Mri Brain Routine
Sex: Female	Modality Name: MR
Report Date: 26/04/2026 06:34:44	

MRI OF THE BRAIN**MRI TECHNIQUE:**

- Sagittal: T1WI 6 mm sections (Plate 1).
- Axial: T2 & FLAIR WI 5 mm sections (Plates 2 & 3).
- Coronal: T1WI images 5 mm sections (Plate 4).
- Post Gd- DTPA sagittal, axial & coronal T1 WI (Plates 5, 6 & 7).

MRI FINDINGS:

- One left frontoparietal and two right temporoparietal intraaxial small ill-defined heterogeneously enhancing likely metastatic SOLs measuring 2.2x2.1x1.6, 1.6x1.1 and 0.9x0.9 cm respectively seen isointense to the cerebral cortex in T1, T2 and FLAIR with no evidence of corresponding diffusion restriction
- The lesions are seen surrounded by grade II vasogenic edema which shows relative diffusion restriction.
- The lesions and the edema on either side are seen creating mass effect in the form of effacement of the surrounding sulci however additional cerebral edematous changes and mild compression of the ipsilateral lateral ventricle are seen on the left side only.
- Right frontoparietal small ill-defined foci of high T2/FLAIR signals involving the deep white matter
- Normal size and parenchymal signal intensity pattern of the cerebellum, and brain stem.
- Normal size, shape and position of the ventricular system. No signs of hydrocephalus or atrophy
- No shift of the midline structures.
- No extra-axial collections.

OPINION:

- Bilateral few intraaxial brain metastatic SOLs
- Chronic small vessel disease.

Dr// Shereen Reda Tayel(MD)



Patient Name: WALAA MOHAMMED ABDLMOEETY ZAHRAH	
Procedure: Mri Brain Routine	
Modality Name: MR	
Age: 46 Year(s), 11 Month(s)	
Sex: Female	
Report Date: 12/07/2026 15:11:02	

MRI OF THE BRAIN

MRI TECHNIQUE:

- Sagittal: T1WI 6 mm sections (Plate 1).
- Axial: T2 & FLAIR WI 5 mm sections (Plates 2 & 3).
- Coronal: T1WI Images 5 mm sections (Plate 4).
- Post Gd- DTPA ssgittal, axial & coronal T1 WI (Plates 5, 6 & 7).

MRI FINDINGS:

- Left frontoparietal linear subcortical high T2/FLAIR signal of restricted diffusion likely ischemic changes
- Stationary course as regards the size of the previously noted left frontoparietal and two right temporoparietal intraaxial small ill-defined heterogeneously enhancing likely metastatic SOLs measuring 2.2x2.1x1.6, 1.6x1.1 and 0.9x0.9 cm respectively seen isointense to the cerebral cortex in T1, T2 and FLAIR with no evidences of corresponding diffusion restriction
- Stationary course as regards the previously noted surrounding grade II vasogenic edema
- The lesions and the edema on either side are still seen creating mass effect in the form of effacement of the surrounding sulci however additional cerebral edematous changes and mild compression of the ipsilateral lateral ventricle are seen on the left side only.
- Right frontoparietal small ill-defined foci of high T2/FLAIR signals involving the deep white matter
- Normal size and parenchymal signal intensity pattern of the cerebellum, and brain stem
- Normal size, shape and position of the ventricular system. No signs of hydrocephalus or atrophy
- No shift of the midline structures.
- No extra-axial collections.

OPINION:

- Stationary course of the bilateral few interaxial brain metastatic SOLs
- Chronic small vessel disease.

Dr/ Shereen Reda Tayel(MD)

Dr. Sherif Wagdy

د. شريف
واغدي
رئيس
العيادة

Patient's Name: Walaa Mohammed Abdel Maati
Date: 2026-07-08
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

C.T. SCAN OF THE BRAIN WITHOUT I.V. CONTRAST INJECTION REVEALED:

- Left high parietal heterogenous hypodense area is seen measuring 9.9 x 8 mm associated with surrounding edema. together they are seen causing mass effect in the form of effacement of the adjacent cortical sulci and compression of the adjacent lateral ventricle.
- Right pareito-temporal ill defined hypodense area seen causing mass effect in the form of effacement of the adjacent cortical sulci.
- No areas of fresh blood density.
- No midline shift.
- No extra-axial collections.
- Posterior fossa structures show no gross abnormality.

Conclusion:

- Left parietal area of altered CT attenuation with surrounding edema likely SOL for contrast enhanced study
- Right pareito-temporal hypodense area likely edema for contrast enhanced study to exclude underlying SOLs.

Dr.sherif Wagdy

Dr.sherif Wagdy

Visit Number: 69926503883 Client Name:
Patient Name: Walaa Mohamed Abd Referred By Prof:
Age / Sex: 47 year / Female Exam Date: 04 Jun 2026 00:00
Patient ID: 2134790086 Acc: 1969031.docx Printed: 05/06/2026 8:41:58 AM

PT

- No pathologically enlarged FDG-avid axillary lymph nodes. Nonspecific small reactive LNs are seen.

II) Brain:

- Metabolically active FDG-avid heterogeneously enhancing space occupying lesion seen in the left parietal region, cortical and subcortical in location, measuring about 2 x 2.5 cm in max. axial diameter with SUVmax 26.05. Moderate surrounding vasogenic edema is seen causing mass effect in form of indentation of the left lateral ventricle.
- Right parieto-occipital hypodense edema is seen with suspected faintly enhancing juxtatorientorial occipital lobe lesion. Better evaluation and follow up by CE-MRI of the brain is recommended.

III) No other metabolically active (FDG avid) lesions detected all over the surveyed body that would account for distant metastatic deposits.

IV) THE REST OF THE BODY:

- Lungs:
 - ✓ No FDG-avid pulmonary nodules / masses.
 - ✓ Right upper lung lobe subpleural reticulations, post RTH effect.
 - ✓ Bilateral apical reticulations, granulomatous.
 - ✓ Bibasilar atelectatic bands.
- Liver: Moderately enlarged with fatty changes showing normal FDG uptake with no metabolically active FDG avid focal lesions.
- Spleen: average in size showing normal FDG uptake with no metabolically active FDG avid splenic focal lesions.
- Skeleton: No FDG avid lytic or sclerotic osseous lesions. Radio-ablation of the lower cervical and upper thoracic spine.
- SIDE FINDING:
 - Prominent renal pelvises.

V) The rest of PET / CT study appears normal.

VI) Normal physiological tracer biodistribution is seen over the heart, bowel, bone marrow and urinary tract.

5132

Visit Number:	69926503883	Client Name:	
Patient Name:	Waliaa Mohamed Abd	Referred By Prof:	
Age / Sex:	47 year / Female	Exam Date:	04 Jun 2026 00:00
Patient ID:	2134790086	Acc: 1969031.docx	Printed: 05/06/2026 8:41:58 AM

PT

CONCLUSION:

PET/CT study for a case of right breast cancer (post-surgery and therapy) showing:

- Clear mastectomy bed. No metabolically active (FDG avid) loco-regional tumoral recurrence.
- Metabolically active FDG-avid cerebral metastatic lesions. Further CE-MRI of the head is advised for better assessment of the brain lesions.
- No other FDG-avid distant metastases.

Prof. Ahmed Wafaie MD, FRCR
Dr. Sally Abdelaziz, MSc

5/3/24

Visit Number:	69926503883	Client Name:	
Patient Name:	Walaa Mohamed Abd	Referred By Prof:	
Age / Sex:	47 year / Female	Exam Date:	04 Jun 2026 00:00
Patient ID:	2134790086	Acc: 1969031.docx	Printed: 05/06/2026 8:41:58 AM

PT

Whole Body PET/CT Examination

HISTORY AND CLINICAL DATA: -

PATIENT DATA: -

- Female patient, 47 years old, Weight: 79 Kg, Blood Glucose level: 120 mg/dl.

RADIOPHARMACEUTICAL: -

- F-18-FDG 4.7 mCi injected I.V.

CLINICAL BACKGROUND: -

- Right breast carcinoma, underwent MRM received CTH & RTH.
- Brain metastasis, received 10 sessions of RTH and now kept on oral CTH.

PURPOSE OF THE STUDY: -

- Assessment.
- No previous study available for comparison.

TECHNIQUE: -

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~2.49 (max. SUV~3.7).

REPORT: -

Review of the PET, CT and the fused images revealed-

I) Breasts and axillae:

a) Right side:

- Status post radical mastectomy showing clear operative bed with no metabolically active (FDG avid) local tumoral recurrence or axillary lymphadenopathy.

b) Left side:

- No FDG-avid breast soft tissue mass lesion detected.